SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Pembrolizumab · every 3 weeks · melanoma before and after surgery

Selected stage IIIB melanoma eligible for the NCCP adjuvant indication: the treating clinician may choose pembrolizumab 200 mg before and after surgery. This is a distinct, discretionary perioperative plan. · NCCP 00455 · version 16 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Pembrolizumab is given before and after planned surgery in this selected melanoma pathway. The team explains the expected benefit, uncertainty, surgical plan and how side effects could change the sequence.

Schedule
Pembrolizumab 200 mg every 3 weeks: 3 doses before surgery, then 15 doses after surgery, subject to recovery and clinical review. The team sets the operation date and postoperative restart; do not assume that surgery or a held dose follows the routine three-week calendar.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Pembrolizumab
PD-1 CHECKPOINT IMMUNOTHERAPY

- ☐ Immune inflammation of bowel, lungs, liver or kidneys
- ☐ Thyroid, pituitary, adrenal and glucose problems
- ☐ Skin reactions and rare severe skin injury
- ☐ Rare heart, muscle, neurological and eye inflammation
- ☐ Infusion reactions
- ☐ Some immune effects need long-term treatment
- ☐ Heart, muscle and nerve immune effects can occur together and require urgent assessment
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Discuss the exact diagnosis, treatment intent, expected benefits and uncertainty, suitable alternatives, the regimen-specific risks and monitoring, fertility and pregnancy precautions, and practical burdens. Confirm the treatment phase and route. This guide supports the conversation and does not replace individual consent. Discuss the possibility that heart, muscle and nerve inflammation can occur together and may need urgent hospital treatment. A severe immune reaction may mean treatment must remain stopped even after recovery. Discuss the possibility that heart, muscle and nerve inflammation can occur together and may need urgent hospital treatment. A severe immune reaction may mean treatment must remain stopped even after recovery.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00455 v16.

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